

GLP-1 Titration Schedule

Complete dose titration guide for semaglutide, tirzepatide, and liraglutide with clinical management tips [Danielle Alcalá · nopriorauthorization.com](#)

SEMAGLUTIDE TITRATION – STANDARD PROTOCOL				
Phase	Weeks	Dose	Frequency	Clinical Goal
Initiation	1–4	0.25mg	Once weekly	GI adjustment — this dose has minimal weight loss effect. Do NOT increase if intolerable GI symptoms
Dose 2	5–8	0.5mg	Once weekly	First therapeutic dose. Most patients see appetite reduction here
Dose 3	9–12	1.0mg	Once weekly	Strong appetite suppression. Weight loss accelerating
Dose 4	13–16	1.7mg	Once weekly	Near-maximum dose. Significant weight loss expected
Maintenance	17+	2.4mg (max)	Once weekly	Maximum approved dose for weight management (Wegovy). Individualize
Diabetes dosing	Per MD	0.5–2mg	Once weekly	Ozempic max 2mg for T2D. Different from Wegovy protocol

TIRZEPATIDE TITRATION – STANDARD PROTOCOL				
Phase	Weeks	Dose	Frequency	Clinical Goal
Initiation	1–4	2.5mg	Once weekly	GI adjustment — lowest dose. Not therapeutic for weight loss yet
Dose 2	5–8	5mg	Once weekly	First therapeutic dose. Appetite begins significant reduction
Dose 3	9–12	7.5mg	Once weekly	Strong dual-mechanism effect. Accelerated weight loss
Dose 4	13–16	10mg	Once weekly	High therapeutic dose. Most patients achieve significant results
Dose 5	17–20	12.5mg	Once weekly	Near-maximum dosing
Maintenance	21+	15mg (max)	Once weekly	Maximum approved dose — individualize based on response/tolerance

DOSE HOLD & SLOW TITRATION RULES	
Hold the current dose if	Patient has uncontrolled nausea/vomiting, lost >1% body weight per week for 3+ weeks (too fast — muscle loss risk), or is hospitalized
Slow titration (6 wks per step)	Severe GI side effects, elderly patients, very sensitive patients, or patients on multiple GI-affecting medications
Do NOT increase if	Patient vomiting more than 3x per week, unable to tolerate any oral intake, or experiencing signs of dehydration
Restart after gap	If >2 weeks missed, restart at previous dose or one dose lower. If >4 weeks missed, restart titration from the beginning
Compounded dosing	Same titration schedule applies to compounded semaglutide and tirzepatide. Verify concentration before dosing (mg/mL matters)

MONITORING SCHEDULE		
Timepoint	Labs	Clinical Assessment
Baseline	CMP, CBC, TSH, fasting glucose, HbA1c, lipid panel, UA	Weight, BMI, BP, HR, waist circumference. Review contraindications
Month 1	Weight only	Assess tolerance, GI side effects, dose readiness. Counsel on protein
Month 3	Weight, BP, fasting glucose	Assess weight loss trajectory. Adjust dose. Discuss expectations
Month 6	CMP, HbA1c, lipid panel, weight	Full metabolic panel. Assess dose optimization. Body composition
Annually	Full panel + thyroid	Annual review. Assess continued need. Long-term planning

MANAGING COMMON SIDE EFFECTS			
Side Effect	First-Line Management	Second-Line	When to Escalate
Nausea	Small meals, low-fat diet, inject at bedtime, stay upright 2hrs after eating	Ondansetron 4mg PRN, Zofran ODT	Persistent vomiting >3x/day, unable to tolerate fluids — hold dose
Constipation	Increase water/fiber, MiraLax daily	Dulcolax, magnesium citrate	Abdominal distension or pain — rule out ileus
Diarrhea	BRAT diet, Imodium PRN, hydration	Electrolyte replacement	Persistent >1 week — check stool for infection
GERD / reflux	Elevate HOB, small meals, avoid lying after eating	PPI (omeprazole), H2 blocker	Persistent despite PPI — may need dose reduction
Fatigue early	Normal — reassure. Usually resolves 4–8 weeks	Ensure adequate protein and hydration	Severe fatigue with lightheadedness — check glucose, BP
Hair loss (telogen effluvium)	Adequate protein intake (critical). Biotin supplement	Reassure — typically self-resolving at 3–6 months	Persistent after 6 months — dermatology referral

CRITICAL CONTRAINDICATIONS — SCREEN EVERY PATIENT	
❌	Personal or family history of Medullary Thyroid Carcinoma (MTC) — BLACK BOX WARNING
❌	Personal or family history of Multiple Endocrine Neoplasia type 2 (MEN 2) — BLACK BOX WARNING
❌	Active or history of pancreatitis — contraindicated for both medications
❌	Pregnancy or planning pregnancy — must use effective contraception. Discontinue if pregnant
❌	Severe kidney or liver disease — use caution, may require dose adjustment or avoidance
⚠️	Active eating disorder — assess carefully. GLP-1 medications may worsen restrictive patterns